

# India's trusted Health Test @Home Service



Booking ID : 14918861838

Sample Collection Date : 25/Aug/2025

**Rahul Ashvin Thakkar**

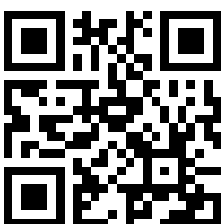
Male, 24 Yrs

## A Comprehensive Health Analysis Report

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AI Based Personalized Report for You

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**INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT**

Check the authenticity of your lab report with machine data

*Scan the QR using any QR code scanner*

## HEALTH ANALYSIS

# Personalized Summary & Vital Parameters

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

Rahul Ashvin Thakkar,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

## 10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

### Your Health Score

81

Out of 100

\*Calculated from test reports



### Thyroid Function

Thyroid Stimulating Hormone (TSH) - Ultrasensitive : 0.95  $\mu$ IU/ml

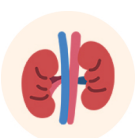
• Everything looks good



### Cholesterol Total

229.3 mg/dl

• Concern



### Kidney Function

Serum Creatinine : 0.59 mg/dl

• Everything looks good



### Vitamin D

44.49 ng/ml

• Everything looks good



### HbA1c

Test not taken



### Vitamin B12

255 pg/ml

• Everything looks good



### Liver Function

Alanine Aminotransferase (ALT/SGPT) : 16 U/L

• Everything looks good



### Calcium Total

9.6 mg/dl

• Everything looks good



### Iron studies

Test not taken



### Complete Hemogram

Haemoglobin (HB) : 14.6 g/dL

• Everything looks good





New Features

## Report Summary

Rahul Ashvin Thakkar

Booking ID 14918861838 | Sample Collection Date: 25/Aug/2025

Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

### Patient Summary Report

Rahul, I understand that receiving test results can sometimes be concerning, but it's important to remember that many factors can influence these values. Let's take a closer look at the areas that need attention and how we can approach them together.

### Summary of Deranged Parameters

Your lipid profile indicates that some cholesterol levels are higher than what is typically considered optimal. This can be influenced by various lifestyle factors, and it's a common issue that many people face. Additionally, your blood count shows some variations that may suggest an immune response or other underlying factors. Lastly, your kidney and liver function tests indicate some values that are slightly outside the normal range, which can often be addressed through lifestyle adjustments.

### Suggestions for Improvement

1.

#### Dietary Adjustments

: Consider incorporating more fruits, vegetables, whole grains, and healthy fats into your diet. Reducing saturated fats and sugars can help improve your cholesterol levels over time.

2.

#### Physical Activity

: Engaging in regular physical activity, such as walking, jogging, or any form of exercise you enjoy, can significantly benefit your overall health and help manage cholesterol levels.

3.

#### Hydration and Nutrition

: Staying well-hydrated and ensuring you have a balanced intake of nutrients can support kidney and liver function. Focus on a diet rich in vitamins and minerals.

4.

#### Regular Monitoring

: Keeping track of your health through regular check-ups can help you stay informed about your progress and make necessary adjustments to your lifestyle.

Remember, small changes can lead to significant improvements in your health. It's great that you're taking the initiative to understand your health better!

### HEALTH ANALYSIS

### HISTORICAL CHARTS

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

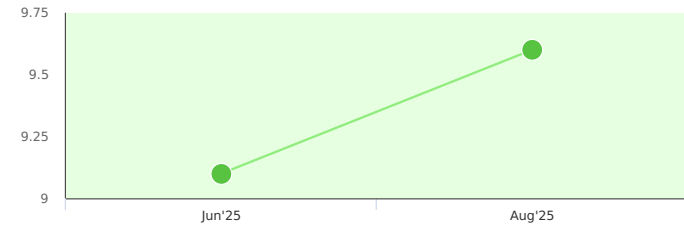
#### Calcium Total, Serum

Everything looks good

Your Latest result

**9.6 mg/dl**

25th Aug 2025



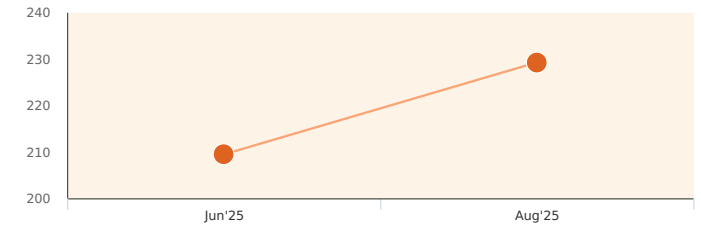
#### Cholesterol-Total, Serum

Concern

Your Latest result

**229.3 mg/dl**

25th Aug 2025



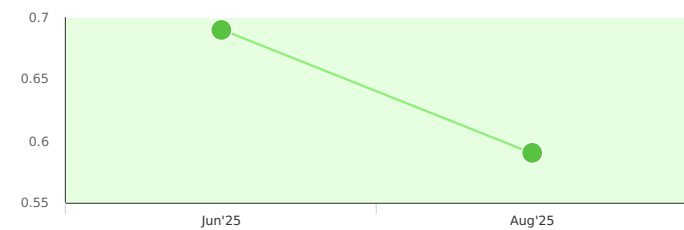
#### Creatinine, Serum

Everything looks good

Your Latest result

**0.59 mg/dl**

25th Aug 2025



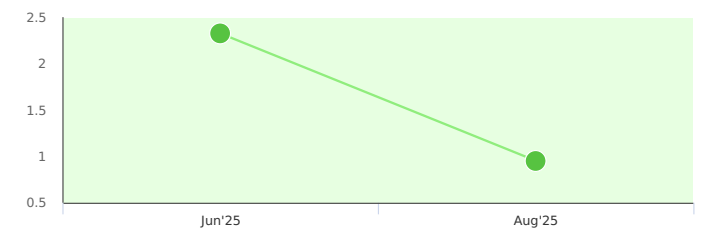
#### TSH Ultra - Sensitive

Everything looks good

Your Latest result

**0.952 µIU/ml**

25th Aug 2025



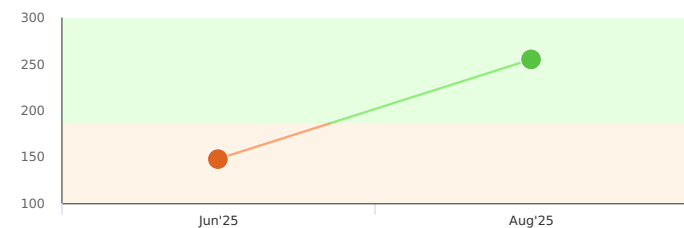
#### Vitamin B12 Cyanocobalamin

Everything looks good

Your Latest result

**255 pg/ml**

25th Aug 2025



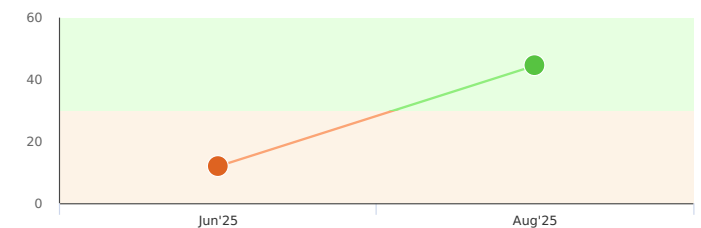
#### Vitamin D Total-25 Hydroxy

Everything looks good

Your Latest result

**44.49 ng/ml**

25th Aug 2025



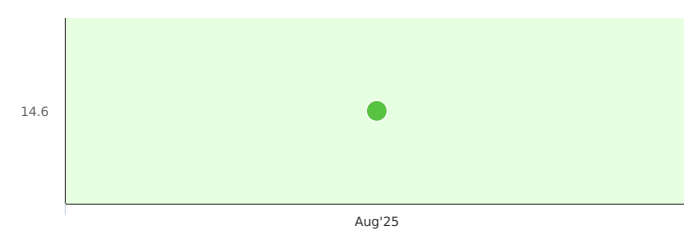
#### Hemoglobin Hb

Everything looks good

Your Latest result

**14.6 g/dL**

25th Aug 2025



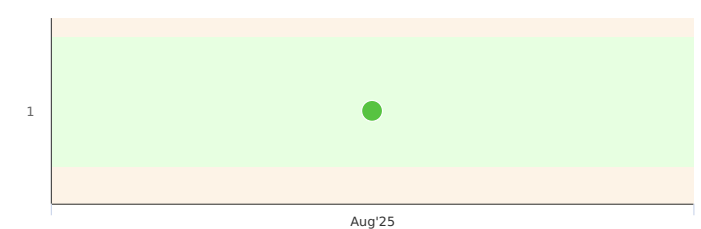
#### SGOT/SGPT Ratio

Everything looks good

Your Latest result

**1 Ratio**

25th Aug 2025



HEALTH ANALYSIS

COMPARATIVE CHARTS

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

↑ Increase From Last Time

↓ Decrease From Last Time

Total Parameters :22  
(As per latest result)

17 Everything Looks Good

1 Borderline

4 Concern

 Reward unlocked!

Better health will now get rewarded, Get rewarded for getting healthier!

|                                                                     |                                                                                   | 2 Months Ago                                      |                                                       |
|---------------------------------------------------------------------|-----------------------------------------------------------------------------------|---------------------------------------------------|-------------------------------------------------------|
| Test Name                                                           | Risk Area                                                                         | Booking ID<br>13973755614<br>07 Jun 2025 08:00 AM | Latest Result<br>25 Aug 2025                          |
| <b>Cholesterol–Total, Serum</b><br>0 –200<br>Normal Range           |  | 209.5<br>mg/dl                                    | ↑ 229.3<br>mg/dl<br>High (Concern)                    |
| <b>HDL Cholesterol Direct</b><br>40 –60<br>Normal Range             | --                                                                                | -                                                 | 41.9<br>mg/dl<br>Everything Looks Good                |
| <b>Triglycerides, Serum</b><br>0 –150<br>Normal Range               |  | -                                                 | 207<br>mg/dl<br>High (Concern)                        |
| <b>Alkaline Phosphatase, Serum</b><br>40 –150<br>Normal Range       | --                                                                                | -                                                 | 75<br>u/L<br>Everything Looks Good                    |
| <b>GGTP (Gamma GT)</b><br>12 –64<br>Normal Range                    | --                                                                                | -                                                 | 20<br>u/L<br>Everything Looks Good                    |
| <b>Proteins, Serum</b><br>6.4 –8.3<br>Normal Range                  | --                                                                                | -                                                 | 7.5<br>g/dl<br>Everything Looks Good                  |
| <b>SGOT/AST</b><br>5 –34<br>Normal Range                            | --                                                                                | 22<br>u/L                                         | ↓ 16<br>u/L<br>Everything Looks Good                  |
| <b>SGPT/ALT</b><br>0 –55<br>Normal Range                            | --                                                                                | 34<br>u/L                                         | ↓ 16<br>u/L<br>Everything Looks Good                  |
| <b>Hemoglobin Hb</b><br>13.0 –17.0<br>Normal Range                  | --                                                                                | -                                                 | 14.6<br>g/dL<br>Everything Looks Good                 |
| <b>Platelet Count Thrombocyte count</b><br>150 –410<br>Normal Range | --                                                                                | -                                                 | 332.5<br>10 <sup>3</sup> /μl<br>Everything Looks Good |
| <b>T3, Total Tri Iodothyronine</b><br>0.6 –1.81<br>Normal Range     | --                                                                                | -                                                 | 1.41<br>ng/ml<br>Everything Looks Good                |
| <b>T4, Total Thyroxine</b><br>3.2 –12.6<br>Normal Range             | --                                                                                | -                                                 | 8.7<br>ug/dl<br>Everything Looks Good                 |

HEALTH ANALYSIS

COMPARATIVE CHARTS

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

↑ Increase From Last Time

↓ Decrease From Last Time

Total Parameters :22  
(As per latest result)

17 Everything Looks Good

1 Borderline

4 Concern

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| Test Name                                               |    | Risk Area | Booking ID<br>13973755614<br>07 Jun 2025 08:00 AM | Latest Result<br>25 Aug 2025              |
|---------------------------------------------------------|----|-----------|---------------------------------------------------|-------------------------------------------|
| TSH Ultra - Sensitive<br>0.35 -5.50<br>Normal Range     | -- |           | 2.32<br>µIU/ml                                    | ↓ 0.95<br>µIU/ml<br>Everything Looks Good |
| Calcium Total, Serum<br>8.4 -10.2<br>Normal Range       | -- |           | 9.1<br>mg/dl                                      | ↑ 9.6<br>mg/dl<br>Everything Looks Good   |
| Chlorides, Serum<br>98 -107<br>Normal Range             | -- |           | -                                                 | 105<br>mmol/L<br>Everything Looks Good    |
| Creatinine, Serum<br>0.2 -1.2<br>Normal Range           | -- |           | 0.69<br>mg/dl                                     | ↓ 0.59<br>mg/dl<br>Everything Looks Good  |
| Phosphorus-Inorganic, Serum<br>2.3 -4.7<br>Normal Range |    |           | -                                                 | 4.9<br>mg/dl<br>High (Concern)            |
| Sodium, Serum<br>136 -145<br>Normal Range               | -- |           | -                                                 | 142<br>mmol/L<br>Everything Looks Good    |
| Urea, Serum<br>19.2 -45<br>Normal Range                 |    |           | 21<br>mg/dl                                       | ↓ 16.8<br>mg/dl<br>Low (Concern)          |
| Uric Acid, Serum<br>3.5 -7.2<br>Normal Range            | -- |           | 6<br>mg/dl                                        | ↓ 5.7<br>mg/dl<br>Everything Looks Good   |
| URINE KETONE<br>Negative -Negative<br>Normal Range      | -- |           | -                                                 | Negative                                  |
| Vitamin D Total-25 Hydroxy<br>30 -100<br>Normal Range   | -- |           | 11.95<br>ng/ml                                    | ↑ 44.49<br>ng/ml<br>Everything Looks Good |

|                |                        |                     |                       |                                                                                     |
|----------------|------------------------|---------------------|-----------------------|-------------------------------------------------------------------------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |  |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |                                                                                     |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 12:32PM |                                                                                     |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 01:00PM |                                                                                     |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |                                                                                     |
| Sample Type    | : Flouride Plasma      | Report Status       | : Final Report        |                                                                                     |

## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

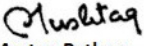
| Test Name                              | Value | Unit  | Bio. Ref Interval |
|----------------------------------------|-------|-------|-------------------|
| <b>Fasting Blood Sugar</b>             |       |       |                   |
| Glucose, Fasting                       | 92.23 | mg/dl | 70 - 100          |
| Method: Enzymatic (Hexokinase/G-6-PDH) |       |       |                   |
| Machine: Abbott ALINITY CI             |       |       |                   |

#### American Diabetes Association Reference Range :

Normal : < 100 mg/dl  
 Impaired fasting glucose(Prediabetes) : 100 - 126 mg/dl  
 Diabetes : >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis

A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with: Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation.

  
 Dr. Mustaq Pathan  
 MBBS, DCP, Pathologist  
 Reg. No: G-29980, Healthians Labs

  
 SIN No: E3049002



Patient Name : Rahul Ashvin Thakkar  
Age/Gender : 24Y 0M 0D /Male  
Order Id : 14918861838  
Referred By : Self  
Customer Since : 25/Aug/2025  
Sample Type : Serum

Barcode : E3049002  
Sample Collected On : 25/Aug/2025 09:16AM  
Sample Received On : 25/Aug/2025 12:36PM  
Report Generated On : 25/Aug/2025 02:31PM  
Sample Temperature : Maintained ✓  
Report Status : Final Report



## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                                                                                       | Value | Unit  | Bio. Ref Interval |
|---------------------------------------------------------------------------------------------------------------------------------|-------|-------|-------------------|
| <b>Liver Function Test (LFT)</b>                                                                                                |       |       |                   |
| Serum Bilirubin, (Total)<br>Method: Diazo<br>Machine: Abbott ALINITY CI                                                         | 0.30  | mg/dl | 0.3 - 1.2         |
| Serum Bilirubin, (Direct)<br>Method: Diazo<br>Machine: Abbott ALINITY CI                                                        | 0.10  | mg/dl | 0.0 - 0.5         |
| Serum Bilirubin, (Indirect)<br>Method: Calculated                                                                               | 0.20  | mg/dl | 0.0 - 0.8         |
| Aspartate Aminotransferase (AST/SGOT)<br>Method: Enzymatic<br>Machine: Abbott ALINITY CI                                        | 16.00 | U/L   | 5 - 34            |
| Alanine Aminotransferase (ALT/SGPT)<br>Method: Enzymatic<br>Machine: Abbott ALINITY CI                                          | 16    | U/L   | 0 - 55            |
| Alkaline Phosphatase (ALP)<br>Method: Para-nitrophenyl Phosphate<br>Machine: Abbott ALINITY CI                                  | 75.00 | U/L   | 40 - 150          |
| Gamma Glutamyl Transferase (GGT)<br>Method: L-Gamma-glutamyl-3-carboxy-4-Nitroanilide<br>substrat<br>Machine: Abbott ALINITY CI | 20.0  | U/L   | 12 - 64           |
| Serum Total Protein<br>Method: Biure<br>Machine: Abbott ALINITY CI                                                              | 7.50  | g/dl  | 6.4 - 8.3         |
| Serum Albumin<br>Method: Bromocresol Green<br>Machine: Abbott ALINITY CI                                                        | 4.85  | g/dl  | 3.5 - 5.2         |
| Serum Globulin<br>Method: Calculated                                                                                            | 2.65  | gm/dl | 3.0 - 4.2         |
| Albumin/Globulin Ratio<br>Method: Calculated                                                                                    | 1.83  | Ratio | 1.2 - 2.5         |
| SGOT/SGPT Ratio<br>Method: Calculated                                                                                           | 1.00  | Ratio | 0.7 - 1.4         |

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones

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SIN No:E3049002

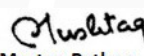
|                |                        |                     |                       |                                                                                     |
|----------------|------------------------|---------------------|-----------------------|-------------------------------------------------------------------------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |  |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |                                                                                     |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 12:36PM |                                                                                     |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 02:31PM |                                                                                     |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |                                                                                     |
| Sample Type    | : Serum                | Report Status       | : Final Report        |                                                                                     |

## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Value | Unit | Bio. Ref Interval |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|------|-------------------|
| getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |       |      |                   |
| AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. Ast levels may also increase after a heart attck or strenuous activity. ALT is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, paget's disease, Rickets, Sarcoidosis etc.                                                                                                                                                                                                                                                                                                                                                                                                |       |      |                   |
| Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-including drugs etc.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |       |      |                   |
| Serum total protein, also known as total protein, is a biochemical test for measuring the total amount of protein in serum..Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma,Waldenstrom's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic - Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver.Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc. |       |      |                   |

  
SIN No:E3049002

  
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Patient Name : Rahul Ashvin Thakkar  
Age/Gender : 24Y 0M 0D /Male  
Order Id : 14918861838  
Referred By : Self  
Customer Since : 25/Aug/2025  
Sample Type : SERUM

Barcode : E3049002  
Sample Collected On : 25/Aug/2025 09:16AM  
Sample Received On : 25/Aug/2025 12:36PM  
Report Generated On : 25/Aug/2025 02:31PM  
Sample Temperature : Maintained ✓  
Report Status : Final Report

## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                                          | Value  | Unit          | Bio. Ref Interval |
|------------------------------------------------------------------------------------|--------|---------------|-------------------|
| <b>Kidney Function Test1 (KFT1)</b>                                                |        |               |                   |
| Serum Creatinine<br>Method: Kinetic Alkaline Picrate<br>Machine: Abbott ALINITY CI | 0.59   | mg/dl         | 0.2 - 1.2         |
| GFR, ESTIMATED<br>Method: Calculated (CKD-EPI)                                     | 138.95 | mL/min/1.73m2 |                   |
| Serum Uric Acid<br>Method: Uricase<br>Machine: Abbott ALINITY CI                   | 5.7    | mg/dl         | 3.5 - 7.2         |
| Serum Calcium<br>Method: Arsenazo III<br>Machine: Abbott ALINITY CI                | 9.6    | mg/dl         | 8.4 - 10.2        |
| Serum Phosphorus<br>Method: Phosphomolybdate<br>Machine: Abbott ALINITY CI         | 4.9    | mg/dl         | 2.3 - 4.7         |
| Serum Sodium<br>Machine: Abbott ALINITY CI                                         | 142    | mmol/L        | 136 - 145         |
| Serum Chloride<br>Method: ISE (Indirect)<br>Machine: Abbott ALINITY CI             | 105    | mmol/L        | 98 - 107          |
| Blood Urea<br>Method: Urease<br>Machine: Abbott ALINITY CI                         | 16.8   | mg/dl         | 19.2 -45          |
| Blood Urea Nitrogen (BUN)<br>Method: Urease                                        | 7.8    | mg/dl         | 8.9 - 20.6        |
| Bun/Creatinine Ratio<br>Method: Calculated                                         | 13.31  | Ratio         |                   |
| Urea/Creatinine Ratio<br>Method: Calculated                                        | 28.47  | Ratio         |                   |

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.

Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout.

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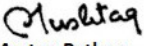
SIN No:E3049002

|                |                        |                     |                       |                                                                                     |
|----------------|------------------------|---------------------|-----------------------|-------------------------------------------------------------------------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |  |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |                                                                                     |
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| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 02:31PM |                                                                                     |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |                                                                                     |
| Sample Type    | : SERUM                | Report Status       | : Final Report        |                                                                                     |

## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | Value | Unit | Bio. Ref Interval |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|------|-------------------|
| <p>Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR .</p> <p>Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells &amp; the balance of chemicals in the body called acids and bases.</p> <p>Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.</p> |       |      |                   |

  
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SIN No: E3049002



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Patient Name : Rahul Ashvin Thakkar  
Age/Gender : 24Y 0M 0D /Male  
Order Id : 14918861838  
Referred By : Self  
Customer Since : 25/Aug/2025  
Sample Type : SERUM

Barcode : E3049002  
Sample Collected On : 25/Aug/2025 09:16AM  
Sample Received On : 25/Aug/2025 12:36PM  
Report Generated On : 25/Aug/2025 02:31PM  
Sample Temperature : Maintained ✓  
Report Status : Final Report

## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                                                      | Value  | Unit  | Bio. Ref Interval                                                                                                                                                                |
|------------------------------------------------------------------------------------------------|--------|-------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Lipid Profile</b>                                                                           |        |       |                                                                                                                                                                                  |
| Total Cholesterol<br>Method: Enzymatic<br>Machine: Abbott ALINITY CI                           | 229.3  | mg/dl | Desirable : <200<br>Borderline: 200-239<br>High : >=240                                                                                                                          |
| Serum Triglycerides<br>Method: GPO-POD<br>Machine: Abbott ALINITY CI                           | 207.0  | mg/dl | Child (1-14 years)<br>Desirable <170mg/dl<br>Borderline 170-199 mg/dl<br>High >199 mg/dl<br>Desirable : <150<br>Borderline high : 150-199<br>High : 200-499<br>Very high : > 500 |
| Serum HDL Cholesterol<br>Method: Accelerator Selective Detergent<br>Machine: Abbott ALINITY CI | 41.9   | mg/dl | 40 - 60                                                                                                                                                                          |
| LDL Cholesterol Calculated<br>Method: Calculated                                               | 146.05 | mg/dl | Optimal : <100<br>near /above Optimal:100 - 129<br>Borderline High: 130- 159<br>High : 160 - 189<br>Very High :>=190                                                             |
| VLDL Cholesterol Calculated<br>Method: Calculated                                              | 41.4   | mg/dl | <30                                                                                                                                                                              |
| Total CHOL / HDL Cholesterol Ratio<br>Method: Calculated                                       | 5.48   | Ratio | 3.30 - 4.40                                                                                                                                                                      |
| LDL / HDL Cholesterol Ratio<br>Method: Calculated                                              | 3.49   | Ratio | Desirable/Low Risk: 0.5-3.0<br>Line/Moderate Risk: 3.0-6.0<br>Elevated/High Risk: >6.0                                                                                           |
| HDL / LDL Cholesterol Ratio<br>Method: Calculated                                              | 0.29   | Ratio | Optimal->0.4<br>Moderate-0.4 to 0.3<br>High-<0.3                                                                                                                                 |
| Non-HDL Cholesterol<br>Method: Calculated                                                      | 187.4  | mg/dl | 0.0 - 160.0                                                                                                                                                                      |

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SIN No:E3049002

|                |                        |                     |                       |                                                                                     |
|----------------|------------------------|---------------------|-----------------------|-------------------------------------------------------------------------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |  |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |                                                                                     |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 12:36PM |                                                                                     |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 02:31PM |                                                                                     |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |                                                                                     |
| Sample Type    | : SERUM                | Report Status       | : Final Report        |                                                                                     |

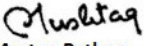
## DEPARTMENT OF BIOCHEMISTRY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | Value | Unit | Bio. Ref Interval |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------|------|-------------------|
| <p>Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations.</p> <p>Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone, vitamin D, estrogens, glucocorticoids and mineralocorticoids.</p> <p>HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease. LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary heart disease.</p> <p>Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. Healthians labs report biological reference intervals (normal ranges) in accordance with the recommendations of The National Cholesterol Education Program (NCEP) &amp; Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.</p> |       |      |                   |



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|                |                        |                     |                       |                                                                                     |
|----------------|------------------------|---------------------|-----------------------|-------------------------------------------------------------------------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |  |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |                                                                                     |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 01:04PM |                                                                                     |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 01:46PM |                                                                                     |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |                                                                                     |
| Sample Type    | : URINE                | Report Status       | : Final Report        |                                                                                     |

## DEPARTMENT OF CLINICAL PATHOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name | Value | Unit | Bio. Ref Interval |
|-----------|-------|------|-------------------|
|-----------|-------|------|-------------------|

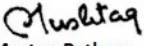
#### Urine Routine & Microscopy Extended

##### PHYSICAL EXAMINATION

|                |             |    |             |
|----------------|-------------|----|-------------|
| Colour         | Pale Yellow |    | Pale Yellow |
| Method: Visual |             |    |             |
| Volume         | 20.00       | mL |             |
| Method: Visual |             |    |             |
| Appearance     | Clear       |    | Clear       |
| Method: Visual |             |    |             |

##### CHEMICAL EXAMINATION

|                                     |          |  |               |
|-------------------------------------|----------|--|---------------|
| Specific Gravity                    | 1.025    |  | 1.001 - 1.035 |
| Method: Dipstick-Ion exchange       |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| pH                                  | 6.0      |  | 4.5 - 7.5     |
| Method: Dipstick-Double indicator   |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Glucose                             | Negative |  | Negative      |
| Method: Dipstick-oxidase peroxidase |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Urine Protein                       | Negative |  | Negative      |
| Method: Dipstick-Bromophenol blue   |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Ketones                             | Negative |  | Negative      |
| Method: Sodium nitroprusside        |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Urobilinogen                        | Normal   |  | Normal        |
| Method: Dipstick-Ehrlichs Test      |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Bilirubin                           | Negative |  | Negative      |
| Method: Dipstick-Ehrlichs Test      |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Nitrite                             | Negative |  | Negative      |
| Method: Dipstick-Griess test        |          |  |               |
| Machine: Uriplus 600                |          |  |               |
| Blood                               | Negative |  | Nil           |
| Method: Dipstick-Peroxidase         |          |  |               |
| Machine: Uriplus 600                |          |  |               |

  
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 SIN No: E3049002

|                |                        |                     |                       |
|----------------|------------------------|---------------------|-----------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 01:04PM |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 01:46PM |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |
| Sample Type    | : URINE                | Report Status       | : Final Report        |

## DEPARTMENT OF CLINICAL PATHOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                       | Value    | Unit | Bio. Ref Interval |
|---------------------------------|----------|------|-------------------|
| Leucocyte Esterase              | Negative |      | Nil               |
| Method: Dipstick- Esterase      |          |      |                   |
| Machine: Uriplus 600            |          |      |                   |
| <b>MICROSCOPIC EXAMINATION</b>  |          |      |                   |
| Pus Cells                       | 1-2      | /HPF | 0 - 5             |
| Method: Microscopic Examination |          |      |                   |
| Epithelial cells                | 1-2      | /HPF | 0 - 5             |
| Method: Microscopic Examination |          |      |                   |
| RBCs                            | Nil      | /HPF | Nil               |
| Method: Microscopic Examination |          |      |                   |
| Casts                           | Nil      |      | Nil               |
| Method: Microscopic Examination |          |      |                   |
| Crystals                        | Nil      |      | Nil               |
| Method: Microscopic Examination |          |      |                   |
| Bacteria                        | Absent   |      | Absent            |
| Method: Microscopic Examination |          |      |                   |
| Yeast Cell                      | Absent   |      | Absent            |

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra. Ketouria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

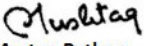
Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.

  
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SIN No: E3049002

Patient Name : Rahul Ashvin Thakkar  
Age/Gender : 24Y 0M 0D /Male  
Order Id : 14918861838  
Referred By : Self  
Customer Since : 25/Aug/2025  
Sample Type : WHOLE BLOOD EDTA

Barcode : E3049002  
Sample Collected On : 25/Aug/2025 09:16AM  
Sample Received On : 25/Aug/2025 12:30PM  
Report Generated On : 25/Aug/2025 01:05PM  
Sample Temperature : Maintained ✓  
Report Status : Final Report



## DEPARTMENT OF HAEMATOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                                                             | Value  | Unit                | Bio. Ref Interval |
|-----------------------------------------------------------------------|--------|---------------------|-------------------|
| <b>Complete Blood Count</b>                                           |        |                     |                   |
| Haemoglobin (HB)                                                      | 14.6   | g/dL                | 13.0-17.0         |
| Method: Photometric Measurement<br>Machine: BECKMAN COULTER DxH560    |        |                     |                   |
| Total Leucocyte Count (TLC)                                           | 7.7    | 10 <sup>3</sup> /uL | 4.0-10.0          |
| Method: Coulter Principle<br>Machine: BECKMAN COULTER DxH560          |        |                     |                   |
| Hematocrit (PCV)                                                      | 44.7   | %                   | 40.0-50.0         |
| Method: Calculated<br>Machine: BECKMAN COULTER DxH560                 |        |                     |                   |
| Red Blood Cell Count (RBC)                                            | 5.41   | 10 <sup>6</sup> /μl | 4.50-5.50         |
| Method: Coulter Principle<br>Machine: BECKMAN COULTER DxH560          |        |                     |                   |
| Mean Corp Volume (MCV)                                                | 82.7   | fL                  | 83.0-101.0        |
| Method: Derived from RBC Histogram<br>Machine: BECKMAN COULTER DxH560 |        |                     |                   |
| Mean Corp Hb (MCH)                                                    | 27.0   | pg                  | 27.0-32.0         |
| Method: Calculated<br>Machine: BECKMAN COULTER DxH560                 |        |                     |                   |
| Mean Corp Hb Conc (MCHC)                                              | 32.6   | g/dL                | 31.5-34.5         |
| Method: Calculated<br>Machine: BECKMAN COULTER DxH560                 |        |                     |                   |
| RDW - CV                                                              | 15.0   | %                   | 11.6-14.0         |
| Method: Derived from RBC Histogram<br>Machine: BECKMAN COULTER DxH560 |        |                     |                   |
| RDW - SD                                                              | 43.70  | fL                  | 39.0-46.0         |
| Method: Derived from RBC Histogram<br>Machine: BECKMAN COULTER DxH560 |        |                     |                   |
| Mentzer Index                                                         | 15.29  | Ratio               |                   |
| Method: Calculated                                                    |        |                     |                   |
| RDWI                                                                  | 229.30 | Ratio               |                   |
| Method: Calculated                                                    |        |                     |                   |
| Green and king index                                                  | 70     | Ratio               |                   |
| Method: Calculated                                                    |        |                     |                   |
| <b>Differential Leucocyte Count</b>                                   |        |                     |                   |
| Neutrophils                                                           | 51.0   | %                   | 40 - 80           |
| Method: Optical/Impedence                                             |        |                     |                   |

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SIN No:E3049002

Patient Name : Rahul Ashvin Thakkar  
Age/Gender : 24Y 0M 0D /Male  
Order Id : 14918861838  
Referred By : Self  
Customer Since : 25/Aug/2025  
Sample Type : WHOLE BLOOD EDTA

Barcode : E3049002  
Sample Collected On : 25/Aug/2025 09:16AM  
Sample Received On : 25/Aug/2025 12:30PM  
Report Generated On : 25/Aug/2025 01:05PM  
Sample Temperature : Maintained ✓  
Report Status : Final Report



## DEPARTMENT OF HAEMATOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name                          | Value | Unit                | Bio. Ref Interval |
|------------------------------------|-------|---------------------|-------------------|
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Lymphocytes                        | 32    | %                   | 20-40             |
| Method: Optical/Impedence          |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Monocytes                          | 8.0   | %                   | 02 - 10           |
| Method: Optical/Impedence          |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Eosinophils                        | 9.0   | %                   | 01 - 06           |
| Method: Optical/Impedence          |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Basophils                          | 0.0   | %                   | 00 - 02           |
| Method: Optical/Impedence          |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| <b>Absolute Leucocyte Count</b>    |       |                     |                   |
| Absolute Neutrophil Count (ANC)    | 3.92  | 10 <sup>3</sup> /uL | 2.0-7.0           |
| Method: Calculated                 |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Absolute Lymphocyte Count (ALC)    | 2.46  | 10 <sup>3</sup> /uL | 1.0-3.0           |
| Method: Calculated                 |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Absolute Monocyte Count            | 0.61  | 10 <sup>3</sup> /uL | 0.2-1.0           |
| Method: Calculated                 |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Absolute Eosinophil Count (AEC)    | 0.69  | 10 <sup>3</sup> /uL | 0.02-0.5          |
| Method: Calculated                 |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| Platelet Count(PLT)                | 332.5 | 10 <sup>3</sup> /μl | 150-410           |
| Method: Coulter Principle          |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |
| MPV                                | 8.1   | fL                  | 7 - 9             |
| Method: Derived from PLT Histogram |       |                     |                   |
| Machine: BECKMAN COULTER DxH560    |       |                     |                   |

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely

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SIN No:E3049002

|                |                        |                     |                       |                                                                                     |
|----------------|------------------------|---------------------|-----------------------|-------------------------------------------------------------------------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |  |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |                                                                                     |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 12:30PM |                                                                                     |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 01:05PM |                                                                                     |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |                                                                                     |
| Sample Type    | : WHOLE BLOOD EDTA     | Report Status       | : Final Report        |                                                                                     |

## DEPARTMENT OF HAEMATOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name | Value | Unit | Bio. Ref Interval |
|-----------|-------|------|-------------------|
|-----------|-------|------|-------------------|

causes, making It necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron-deficiency anaemia is more likely.

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|                |                        |                     |                       |
|----------------|------------------------|---------------------|-----------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 12:36PM |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 01:20PM |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |
| Sample Type    | : Serum                | Report Status       | : Final Report        |

## DEPARTMENT OF IMMUNOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name | Value | Unit | Bio. Ref Interval |
|-----------|-------|------|-------------------|
|-----------|-------|------|-------------------|

#### Vitamin B12

|                             |     |       |           |
|-----------------------------|-----|-------|-----------|
| VITAMIN B12                 | 255 | pg/ml | 211 - 911 |
| Method: CLIA                |     |       |           |
| Machine: Siemens Centaur XP |     |       |           |

Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine, and 2) the conversion of methylmalonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. Also causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

#### Vitamin D, 25-Hydroxy

|                               |       |       |          |
|-------------------------------|-------|-------|----------|
| VITAMIN D (25 - OH VITAMIN D) | 44.49 | ng/ml | 30 - 100 |
| Method: CLIA                  |       |       |          |
| Machine: Siemens Centaur XP   |       |       |          |

| VITAMIN D STATUS | VITAMIN D 25 HYDROXY (ng/mL), Adult | VITAMIN D 25 HYDROXY (ng/mL), Pediatric |
|------------------|-------------------------------------|-----------------------------------------|
| DEFICIENCY       | <20                                 | <15                                     |
| INSUFFICIENCY    | 20 - 30                             | 15 - 20                                 |
| SUFFICIENCY      | 30 - 100                            | 20 - 100                                |

Vitamin D is a lipid-soluble steroid hormone that is produced in the skin through the action of sunlight or is obtained from dietary sources. The role of vitamin D in maintaining homeostasis of calcium and phosphorus is well established. The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs). The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20-30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.

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SIN No: E3049002

|                |                        |                     |                       |
|----------------|------------------------|---------------------|-----------------------|
| Patient Name   | : Rahul Ashvin Thakkar | Barcode             | : E3049002            |
| Age/Gender     | : 24Y 0M 0D /Male      | Sample Collected On | : 25/Aug/2025 09:16AM |
| Order Id       | : 14918861838          | Sample Received On  | : 25/Aug/2025 12:36PM |
| Referred By    | : Self                 | Report Generated On | : 25/Aug/2025 01:48PM |
| Customer Since | : 25/Aug/2025          | Sample Temperature  | : Maintained ✓        |
| Sample Type    | : Serum                | Report Status       | : Final Report        |

## DEPARTMENT OF IMMUNOLOGY

### Healthy India 2025 Full Body Checkup Advance

| Test Name | Value | Unit | Bio. Ref Interval |
|-----------|-------|------|-------------------|
|-----------|-------|------|-------------------|

#### Thyroid Profile (Total T3, T4, TSH)

|                                                  |       |        |             |
|--------------------------------------------------|-------|--------|-------------|
| Tri-Iodothyronine (T3, Total)                    | 1.41  | ng/ml  | 0.6 - 1.81  |
| Method: CLIA<br>Machine: Siemens Centaur XP      |       |        |             |
| Thyroxine (T4, Total)                            | 8.70  | ug/dl  | 3.2-12.6    |
| Method: CLIA<br>Machine: Siemens Centaur XP      |       |        |             |
| Thyroid Stimulating Hormone (TSH)-Ultrasensitive | 0.952 | μIU/ml | 0.35 - 5.50 |
| Method: CLIA<br>Machine: Siemens Centaur XP      |       |        |             |

| Pregnancy interval | Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association) |
|--------------------|-----------------------------------------------------------------------|
| First trimester    | 0.1 - 2.5                                                             |
| Second trimester   | 0.2 - 3.0                                                             |
| Third trimester    | 0.3 - 3.0                                                             |

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

\*\*\* End Of Report \*\*\*

*Mushtaq*  
Dr. Mushtaq Pathan  
MBBS, DCP, Pathologist  
Reg. No: G-29980, Healthians Labs

SIN No: E3049002

#### Terms & Conditions:

- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of  $\pm 50\%$ , hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.
- 13) This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- 14) All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- 15) This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- 16) It is strongly recommended to take required precautions for allergic reactions or sensitivities.
- 17) Authorised partner labs as mentioned for certain tests are as below:
  - HL/PL/001- Metropolis Healthcare Ltd
  - HL/PL/002- Thyrocare technologies Limited
  - HL/PL/003- Lifecell International Pvt. Ltd. - Laboratory Services
  - HL/PL/004- Modern Diagnostic & Research Centre

## ADVISORY

## Health Advisory

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

**32.11** Body Mass Index

**5'7"** Height (ft/in)

**95** Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

0cm



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

## SUGGESTED NUTRITION

### SUGGESTED NUTRITION

#### Do's

- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Include fruits like apples, berries and melons in your diet
- Include fresh garlic and fenugreek seeds in your diet
- Include whole grains in your diet like whole wheat bread and other products, brown rice or hand pounded rice, oats

#### Dont's

- Avoid flavoured and seasoned foods
- Decrease intake of colas and sugary drinks
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Limit sugar intake
- Avoid red meat and organ meats
- Reduce caffeine intake
- Avoid high cholesterol and calorie dense foods
- Limit the use of oil and avoid sauces and dressings

## SUGGESTED LIFESTYLE

### SUGGESTED LIFESTYLE

#### Do's

- Sleep well at night and do relaxing activities
- Maintain ideal weight
- Lose weight gradually and stay active

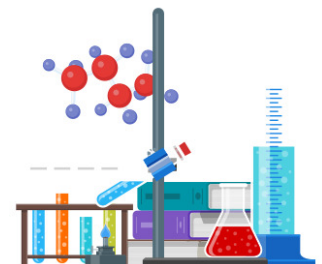
#### Dont's

- Avoid overexertion without having food or drink
- Avoid strenuous exercises
- Avoid smoking and alcohol
- Avoid long periods of inactivity
- Don't ignore your body signals and don't skip your regular health check-ups
- Avoid late night heavy meals
- Avoid overeating or calorie rich food

## SUGGESTED FUTURE TESTS

### SUGGESTED FUTURE TESTS

- Complete Hemogram - **Every 2 Month**
- Peripheral Smear Examination By Pathologist - **Every 2 Month**
- Vitamin D Total-25 Hydroxy - **Every 2 Month**
- Calcium Total, Serum - **Every 2 Month**
- Lipid Profile Advance - **Every 3 Month**
- Liver Function Test - **Every 3 Month**
- HsCRP High Sensitivity CRP - **Every 4 Month**



### HEALTH ADVISORY

### Suggestions for Health & Well-being

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

## PHYSICAL ACTIVITY

### PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



### BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

## BALANCED DIET

## STRESS MANAGEMENT

### STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



### BMI

BMI recommended range is 18.5 to 24.9. Your BMI is **32.11**, which is on a higher side.

Please follow recommended diet and maintain a healthy lifestyle and try to keep your BMI within the desired range. Keeping the right BMI for your body helps prevent many untimely diseases and goes a long way.

#### BMI CHART



## SUGGESTED BMI

### Supplement Suggestions

Rahul Ashvin Thakkar

Booking ID : 14918861838 | Sample Collection Date : 25/Aug/2025

Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

| Deficiency/Out of Range Parameter(s) | Suggested Supplement |
|--------------------------------------|----------------------|
| Cholesterol-Total, Serum             | APPLE CIDER VINEGAR  |

To order, call 1800-572-000-4

### Suggestions for Improving Deficiencies



#### APPLE CIDER VINEGAR

Boosts Energy & Improves Immunity

Herbved Apple Cider Vinegar with Mother is 100% natural, raw, unfiltered and unpasteurized that has been fermented from Himalayan apples. It contains a substance called mother that consists of antioxidants, proteins, enzymes, and friendly bacteria that offers you a host of health benefits like losing weight, controlling High BP, Sugar & Cholesterol and improving digestion. It can work wonders on your hair making it soft and shiny, controlling dandruff & hairfall and can also prevent acne when applied as a toner externally on your skin.

Benefits of Apple Cider Vinegar:

Helps in weight loss | Helps control cholesterol levels | Lowers blood pressure | Controls hairfall and makes hair soft and shiny | Improves digestion



#### IMMUNO-PLUS

Give your immunity a boost the all-natural way.

IMMUNO-PLUS is the perfect all-natural herbal supplement to boost your immune system and strengthens your body's defenses against diseases and infections. IMMUNO-PLUS provides your immune system the necessary reinforcement to keep you safe and healthy.

A weakened immune system opens you to a host of illnesses, such as:

• Recurring Infections | • Heightened Risk of Cancer | • Autoimmune Disorders | • Slow Growth Rate | • Serious Damage to the Heart, Lungs, Digestive Tract & the Nervous System

Infused with the ages-proven goodness of all-natural ingredients, IMMUNO-PLUS is the perfect supplement to strengthen your immune system without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in IMMUNO-PLUS present the following benefits:

##### Amla

Boosts immunity & Stores antioxidants

##### Jetwatika

Antioxidant properties strengthen the immune system

##### Aloe Vera

Fights against oxygenated rogue molecules in the blood

##### Ashwagandha

Reinforces the immune system to increase its fighting ability

##### Ginger

Anti-inflammatory & antioxidant effects reinforce the immune system



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### About Healthians Labs

## How we control Report Accuracy at Healthians



### Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



### Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



### QR Code

QR Code based authenticity check on all its reports



### Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



### Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



### EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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